

Patient Name: [REDACTED]
[REDACTED]

Date: [REDACTED]
[REDACTED]

FOLLOW-UP VISIT

Diagnosis/Problem
Primary Diagnosis(es): Other iron deficiency anemias
Iron deficiency
Malignant neoplasm of sigmoid colon

Date	Type	ICD-9	ICD-10	Description	Disease Status	Status Date
10/20/2022	Secondary	241.1	E04.2	Nontoxic multinodular goiter		

Date	Type	ICD-9	ICD-10	Problem	Comment	Status
11/25/2022	Diagnosis	715.15	M16.10	Unilateral primary osteoarthritis, unspecified hip		Active

Reason for Today's Visit:
8/15/2025: follow up of colon ca

History of Present Illness
[REDACTED] al resection of colon ca c/w T2N0 adenocarcinoma
Patient has had close follow up with colonoscopies every 3 years due to prior history of polyps as well as family history of colon ca in her mother at the age of 70
She reports not smoking daily anymore, last cigarette was 3/9/2022
s/p CT c/a/p - reviewed with pt and husband
stable lung nodules, lymphadenopathy
reviewed findings of abdominal portion including gallstones and diverticulosis
s/p gallbladder removal
s/p PET CT 9/2022: activity noted at suture site, thyroid calcification noted, Multiple prominent peripancreatic and portacaval lymph nodes without associated hypermetabolic activity, Right intraparotid hyper metabolic soft tissue nodule likely benign in etiology
lung nodules small, with no activity

Underwent thyroid ultrasound in October 2022
There are three nodules nodule, one nodule of concern measuring 1.7cm
s/p biopsy 12/2022: reviewed with patient - negative for malignancy

pt reports she has quit smoking, has been almost 1 year.
PET CT 9/2023 - NED.
last CT chest was 3/2022 - multiple pulm nodules.

PET CT 10/2024 - new hepatic foci noted.
MRI abdomen 10/2024 - hepatic mets noted.
patient is s/p liver mass biopsy - c/w metastatic colon ca

has initiated xeloda 11/2024

dealing with chronic memory issues.
PET CT 2/2025 - response to treatment.
PET CT from 5/23/25 reviewed, minute changes that show no progression of disease

August 15th 2025:
has been doing very well
imaging reviewed
PET CT 82/2025 - progression of disease
treatment options discussed

Past medical History
HTN
HLD

Past Surgical History
colon resection
left breast lumpectomy- benign

Social History
Patient lives with spouse. Light tobacco smoker Advised to Quit Denies any prior alcohol use. Denies any illicit drug use.

Family History
colon ca - mother age 70

Personal History
Patient reports having routine colonoscopy. Last done:2021 Patient denies having ever received blood products in the past.

GYN History
Estrogen use was discontinued in appx 1980. Patient has had 3 live births.

Allergies
No Known Drug Allergies

Medications

Inside	Drug	Script Date	Qty	Rfls	Instructions
	Calcium 500 500 mg calcium (1,250 mg) tablet	10/8/2021		0	1 p.o. q. day
Y	capecitabine 500 mg tablet	11/11/2024	84	11	TAKE 3 TABLETS (1500MG) BY MOUTH TWICE DAILY DAYS 1-14 EVERY 21 DAY CYCLE
	doxycycline hyclate 20 mg tablet	10/4/2024		0	pt does not know dosage will be followed up on next visit
Y	Injectafer 50 mg iron/mL intravenous solution	3/16/2022	30	3	INFUSE 750MG IV ON DAYS 1 & 8
Y	Injectafer 50 mg iron/mL intravenous solution	1/20/2025	15	2	INFUSE 750MG INTRAVENOUSLY x 1 dose
	iron ER 159 mg (45 mg iron) tablet,extended release	10/8/2021		0	1 p.o. q. day
	Lipitor 10 mg tablet	10/8/2021		0	1 p.o. q. day
	losartan 25 mg tablet	10/8/2021		0	1 p.o. q. day unsure dose
Y	ondansetron 8 mg disintegrating tablet	11/14/2024	30	0	Place 1 tab on tongue every 6-8 hours prn for nausea

	Paxil 40 mg tablet	3/11/2022		0	1 p.o. q. day
	Vitamin B-12 1,000 mcg tablet	3/11/2022		0	1 p.o. q. day
	Wellbutrin XL 150 mg 24 hr tablet, extended release	10/8/2021		0	450mg 1 p.o. q. day

Review Of Systems

12 point review of systems with positives reported by patient since last visit.

Constitutional:

· Fatigue

Fever : No Fever. Chills No Rigors. No Chills.

Vitals

Vitals on 8/15/2025 10:53:00 AM: Height=64in, Weight=143lb, Temp=97.3f, Pulse=100, SystolicBP=149, DiastolicBP=79, O2 Sat=95%

Physical Exam

Normal presentation. In no acute distress. Not anxious.

Regular Rate and Rhythm

No dyspnea. No rhonchi. No wheezing.

Abdomen is soft and non-tender to touch.

Affect normal.

No edema.

Patient reported pain

Pain Scale 0 - No pain

Treatment Recommendations:

Reassess pain at next visit.

ECOG

Performance Status -ECOG 0: Fully active, able to carry on all pre-disease performance without restriction

Labs

Lab results on 8/15/2025: WBC=6.6 x10³/UL, RBC=4.00 x10⁶/UL, Hgb=13.5 g/dl, HCT=40.6 %, MCV=**101.5** fL, MCH=**33.8** pg, MCHC=33.3 g/dL, RDW [Ratio]=**18.8** %, MPV=8.1 fL, Plat=218 x10³/UL, ANC=4.3 x10³/UL, Lymph#=1.5 x10³/UL, MONO#=0.5 x10³/UL, BASO#=0.0 x10³/UL, EOS#=0.3 x10³/UL, Gran%=64.7 %, Lymph%=22.6 %, MONO%=8.0 %, BASO%=0.5 %, EOS%=3.9 %, Sodium=142 mmol/L, Potassium=4.0 mmol/L, Chloride=103 mmol/L, CO2=25 mmol/L, Glucose=**126** mg/dL, BUN=16.9 mg/dL, Creat=**0.95** mg/dL, BUN Creat Ratio=17.8 calc, Uric Acid=**6.5** mg/dL, Total Protein=7.5 g/dL, Albumin=4.3 g/dL, A/G=1.3 calc, Globulin=3.2 calc, Total Bili=0.8 mg/dL, Alk Phos=135 U/L, ALT=20 U/L, AST=**33** U/L, Phosphorus=**2.9** mg/dL, LDH=**256** U/L, Calcium=9.6 mg/dL, Magnesium=2.1 mg/dL, eGFR (African American)=68.4 mL/min/1.73m, eGFR (Non-Afr. American)=**56.5** mL/min/1.73m, IRON=135 ug/dL, UIBC=256 ug/dL, TIBC=391.0 calc, Iron Sat Percent=35 calc, Ferritin=118.0 ng/mL, B12=402.0 pg/mL, Folate=12.3 ng/mL, CEA=**11.0** ng/mL

Radiology Reports

Print?	Date of Doc.	Name	MD Interpretation	Comment
☐				
☒	8/13/2025	PET CT (SKULL BASE TO MID THIGH)		

Pathology Reports

Assessment:

Recommendation/Plan:

1. Colon ca:

pathology reviewed - **patient with stage 1 colon ca**

discussed that 20 lymph nodes have been removed and they are all negative

Guidelines reviewed at length - for stage 1 colon ca and optimal resection, no role for monitoring of tumor markers on yearly imaging

CEA reviewed - elevation noted likely due to smoking

s/p CT chest - reviewed at length - multiple sub **centimeter** nodules, enlarged hilar lymph nodes

s/p CT Chest, Abdomen and Pelvis in March 2022 with stable portacaval and peripancreatic lymph nodes and sub centimeter

pulmonary nodules and incidental cholelithiasis and diverticulosis reported

s/p PET CT 9/2022: activity noted at suture site

Multiple prominent peripancreatic and portacaval lymph nodes without associated hypermetabolic activity

Right intraparotid hyper metabolic soft tissue nodule likely benign in etiology

lung nodules small, with no activity

patient had colonoscopy done 9/26/22 and now again 3/2023 -

small increase in CEA noted - has been elevated prior and no evidence of malignancy - elevation possibly due to smoking

PET CT 9/2023 - NED.

advised pt to continue to follow with GI. last scope was March 2023. has upcoming appt in Dec 2023.

PET CT 10/2024 - new hepatic foci noted.

MRI abdomen 10/2024 - hepatic mets noted.

biopsy done - c/w metastatic colon ca

xeloda initiated 11/2024.

PET CT 2/2025 - response to treatment.

will monitor lung nodule

recovering well from ankle fracture, now out of rehab

baseline PET CT reviewed from 5/23/25 which showed no significant progression of disease

patient is clinically doing well, main concern is intermittent fatigue, loose stools

remains on Xeloda one week on / one week off

PET CT 8/2025: progression of disease

discussed treatment options

discussed FOLFOX vs xeloda avastin

for now will add avastin to regimen

will repeat imaging in 3 months

nurse ed scheduled

2. Anemia

Slow Fe for iron deficiency taken daily

Worsening depletion of iron stores observed

s/p IV iron

no anemia noted today

encouraged stool softener with oral iron use - will notify if she can stop supplementation.

iron def noted.

cbc stable

follow with iron panel, vitamin levels - replete as needed.

3. Thyroid Nodules

Prior PET on 9/24/2022 nodes hypermetabolic right thyroid nodule

further imaging done on 10/20/2022 with right thyroid lobe measuring up to 1.7cm

biopsy recommended

s/p thyroid biopsy 12/2022: negative for malignant cells

should have every 6 month follow up with PMD

now with elevation in TSH - blood work faxed to PMD - patient to continue with follow up

thyroid US 4/2024 - stable thyroid nodules.

MD follow up 1 month

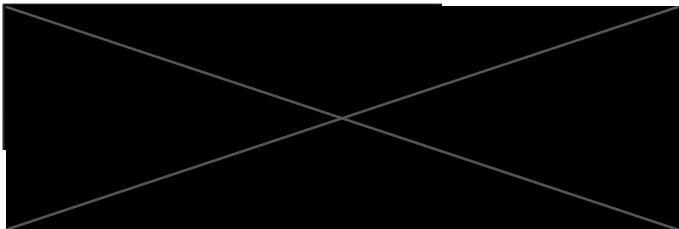
Current Treatment: Order Tests: *CBC/Diff/Platelets, *Folic Acid / Folate, *B-12, *CEA, *Ferritin, *FE/Iron, *Total Iron Binding (TIBC)/UIBC, *Comp Metabolic Profile (CMP), *Magnesium, *LDH, *Uric Acid, *Phosphorus.

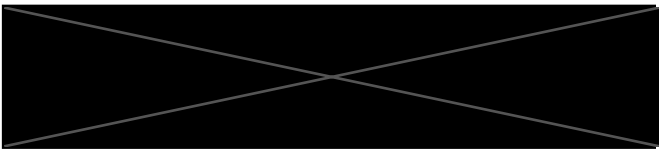
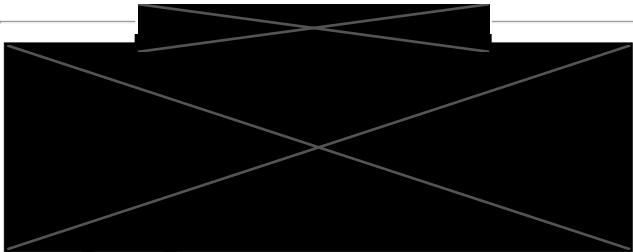
Patients laboratory results were reviewed and explained in full. All questions answered. Radiology imaging and dictated report were reviewed; the disease status and level of response to the treatment were explained in great detail.

More than half of time was spent for providing counseling and coordination of care. The diagnosis and care plan were discussed with e to discuss many aspects of care; questions were answered to patient's satisfaction.

4 weeks.

Signed





Patient Name: [Redacted]

Comments: one exlav two cor in house lg

Result	Value (Previous)	Units	Range	Lab
Vitamin B-12 -Final	Ordered by			
	Comments: Performed at: NYCSLAB Core Medical Laboratories - New York Cancer and Blood Specialists CLIA/CLEP: 33D2141642/9194 1500 Route 112, Bldg 4 Floor 1, Port Jefferson Station, NY,11776			
B12	402.0 (458.0)	pg/mL	232.0 - 1245.0	NYCSLAB~AV



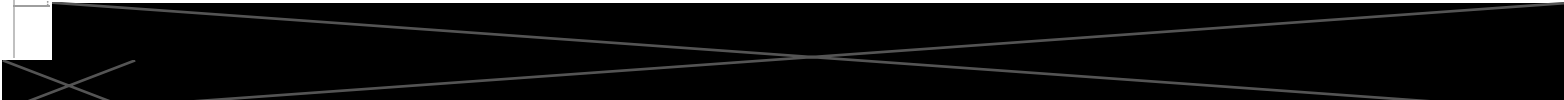
CEA -Final	Ordered by [Redacted]			
CEA	11.0 High (9.6)	ng/mL	0.0 - 4.7	NYCSLAB--AV
CEA Reference Range Note: Nonsmoker: <3.9 Smoker: <5.6 Roche Diagnostics - ECLIA (Electrochemiluminescence Immunoassay) Values obtained with different assay methods or kits cannot be used interchangeably. Results cannot be interpreted as absolute evidence of the presence or absence of malignant disease.				



Comprehensive Metabolic Panel -Final	[Redacted]			
Glucose	126 High (85)	mg/dL	82 - 115	NYCSLAB--AV
BUN	16.9 (19.7)	mg/dL	8.0 - 23.0	NYCSLAB--AV
Creat	0.95 High (0.96)	mg/dL	0.50 - 0.90	NYCSLAB--AV
BUN Creat Ratio	17.8	calc	11.0 - 26.0	NYCSLAB--AV
Sodium	142 (143)	mmol/L	136 - 145	NYCSLAB--AV
Potassium	4.0 (3.5)	mmol/L	3.5 - 5.1	NYCSLAB--AV
Chloride	103	mmol/L	98 - 107	NYCSLAB--AV
CO2	25	mmol/L	22 - 29	NYCSLAB--AV
Calcium	9.6 (10.0)	mg/dL	8.6 - 10.4	NYCSLAB--AV
Total Protein	7.5 (7.3)	g/dL	6.6 - 8.7	NYCSLAB--AV
Albumin	4.3 (4.4)	g/dL	3.5 - 5.2	NYCSLAB--AV
Globulin	3.2	calc	2.5 - 3.5	NYCSLAB--AV
A/G	1.3	calc	1.2 - 2.2	NYCSLAB--AV
Alk Phos	135 (131)	U/L	35 - 140	NYCSLAB--AV
Total Bili	0.8 (0.9)	mg/dL	0.0 - 1.2	NYCSLAB--AV
ALT	20 (14)	U/L	0 - 33	NYCSLAB--AV
AST	33 High (26)	U/L	0 - 32	NYCSLAB--AV
eGFR (African American)	68.4	mL/min/1.73m	>60.0	NYCSLAB--AV
eGFR (Non-Afr. American)	56.5 Low	mL/min/1.73m	>60.0	NYCSLAB--AV
Corrected Serum Calcium	Nonapplicable	mg/dL		NYCSLAB--AV



Ferritin -Final	Ordered by: [Redacted]			
Ferritin	118.0 (112.0)	ng/mL	15.0 - 150.0	NYCSLAB--AV



Folate -Final					
Folate	12.3 (4.4)	ng/mL	>3.0	NYCSLAB--AV	

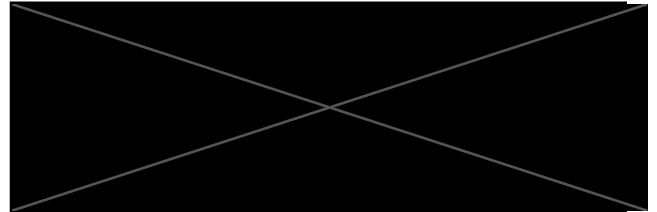
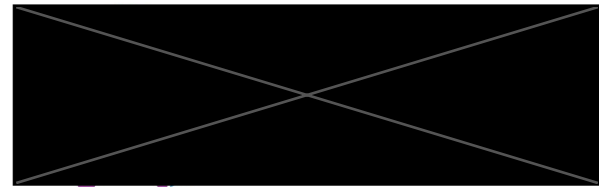
Iron Panel -Final <i>Ordered by: Adriana Guigova, DO</i>					
IRON	135 (117)	ug/dL	37 - 145	NYCSLAB--AV	
TIBC	391.0	calc	245.0 - 400.0	NYCSLAB--AV	
UIBC	256	ug/dL	112 - 347	NYCSLAB--AV	
Iron Sat Percent	35	calc	12 - 50	NYCSLAB--AV	

LDH -Final <i>Ordered by: Adriana Guigova, DO</i>					
LDH	256 High (221)	U/L	135 - 214	NYCSLAB--AV	

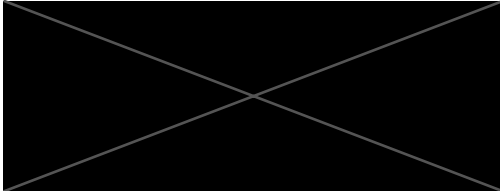
Magnesium -Final <i>Ordered by: Adriana Guigova, DO</i>					
Magnesium	2.1 (2.0)	mg/dL	1.6 - 2.4	NYCSLAB--AV	

Phosphorus -Final <i>Ordered by: Adriana Guigova, DO</i>					
Phosphorus	2.9 Low (3.0)	mg/dL	3.0 - 4.3	NYCSLAB--AV	

Uric Acid -Final <i>Ordered by: Adriana Guigova, DO</i>					
Uric Acid	6.5 High (5.9)	mg/dL	2.4 - 5.7	NYCSLAB--AV	



Patient Name:
Patient Number
Report Date:
Laboratory Use Onl



DOB: 6/16/1944
Sex: Female



Comments:

	Result	Value (Previous)	Units	Range	Lab
CBC -Final <i>Ordered by: Adriana Guigova, DO</i>					
	WBC	6.6 (10.5)	x10 ³ /UL	3.8 - 10.8	SMO235-M.H
	RBC	4.00	x10 ⁶ /UL	3.80 - 5.80	SMO235-M.H
	Hgb	13.5 (13.4)	g/dl	11.5 - 16.0	SMO235-M.H
	HCT	40.6 (41.3)	%	37.0 - 47.0	SMO235-M.H
	MCV	101.5 <i>High</i>	fL	80.0 - 100.0	SMO235-M.H
	MCH	33.8 <i>High</i>	pg	27.0 - 33.0	SMO235-M.H
	MCHC	33.3	g/dL	32.0 - 36.0	SMO235-M.H
	RDW [Ratio]	18.8 <i>High</i>	%	11.0 - 16.0	SMO235-M.H
	MPV	8.1	fL	6.0 - 12.5	SMO235-M.H
	Plat	218 (234)	x10 ³ /UL	150 - 500	SMO235-M.H
	Gran%	64.7	%	38.0 - 80.0	SMO235-M.H
	Lymph%	22.6	%	15.0 - 51.1	SMO235-M.H
	MONO%	8.0	%	0.0 - 13.0	SMO235-M.H
	EOS%	3.9	%	0.0 - 8.0	SMO235-M.H
	BASO%	0.5	%	0.0 - 2.0	SMO235-M.H
	ANC	4.3 (7.6)	x10 ³ /UL	1.5 - 7.8	SMO235-M.H
	Lymph#	1.5	x10 ³ /UL	0.9 - 3.9	SMO235-M.H
	MONO#	0.5	x10 ³ /UL	0.1 - 1.0	SMO235-M.H
	EOS#	0.3	x10 ³ /UL	0.0 - 0.5	SMO235-M.H
	BASO#	0.0	x10 ³ /UL	0.0 - 0.2	SMO235-M.H
<i>Performing Location: SMO235 = Med/Onc - Setauket - 347, 49 Route 347, , Port Jefferson Station NY</i>					
-- End of Report --					

Signed By

